Amerimed
WELLNESS INSTITUTE

Amerimed Wellness Institute
2800 Pacific Crest Boulevard, San Jose, CA 95134
www.amerimedwellness.example

CLINICAL POLICY · AMW-CLN-001

Infection Prevention & Control Policy

Standard and transmission-based precautions, hand hygiene, surveillance and outbreak management

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Policy owner	Dr. Samuel Okafor, MD, MPH, Medical Director, Infection Prevention	**Approved by**	Infection Prevention Committee and Medical Executive Committee
Applies to	All workforce members, licensed independent practitioners, students, contractors and visitors in Amerimed facilities	**Classification**	Internal

1. Purpose and authority

This policy sets out how Amerimed reduces the risk of healthcare-associated infections (HAIs) to patients, staff and visitors. It is based on: - the CMS Conditions of Participation (42 CFR 482.42) - CDC/HICPAC guidelines - the Cal/OSHA Aerosol Transmissible Diseases Standard (Title 8, Section 5199) - the OSHA Bloodborne Pathogens Standard (29 CFR 1910.1030) - California SB 739 / SB 1058 HAI reporting laws - Oregon and Washington HAI reporting rules

The **Infection Prevention Committee** (meets monthly) has authority to institute any control measure when there is a risk of transmission. This includes closing units to admissions.

2. Standard precautions (all patients, all the time)

1. **Hand hygiene:** alcohol-based hand rub (ABHR) or soap and water at the **5 Moments**: before touching a patient, before a clean or aseptic procedure, after body-fluid exposure risk, after touching a patient, and after touching patient surroundings. Soap and water is required for *C. difficile* and norovirus, and when hands are visibly soiled.
2. **PPE** chosen on anticipated exposure (gloves, gown, mask, eye protection).
3. **Respiratory hygiene / cough etiquette:** masks at entrances for symptomatic people.
4. **Safe injection practices:** one needle, one syringe, one patient. Single-dose vials are used for one patient only.

5. **Sharps safety:** engineered safety devices are required. Needles are never recapped.
6. **Environmental cleaning** with EPA-registered disinfectants, following contact times.
7. **Reprocessing of reusable medical equipment** following manufacturer instructions for use (IFU).

3. Transmission-based precautions

Precaution	Examples	Key requirements
Contact	MRSA, VRE, CRE, *C. difficile* (Contact-Plus), scabies	Gown and gloves on room entry; dedicated equipment; sporicidal cleaning for *C. diff*
Droplet	Influenza, pertussis, *N. meningitidis* (first 24 h of therapy), mumps	Surgical mask within 6 feet; private room preferred
Airborne	Tuberculosis, measles, varicella, disseminated zoster	Airborne Infection Isolation Room (AIIR, negative pressure, ≥12 ACH); fit-tested N95 or PAPR
Enhanced (COVID-19 and novel respiratory pathogens)	SARS-CoV-2, novel influenza	N95, eye protection, gown and gloves; AIIR for aerosol-generating procedures
Special pathogens	Ebola, Marburg, MERS	Activate the Special Pathogens Plan; transfer to the regional treatment centre (Amerimed Pacific Medical Center is the designated Assessment Hospital - illustrative)

Isolation signs are posted at the doorway. Precautions are discontinued only by Infection Prevention or under nurse-driven protocols.

4. Hand hygiene monitoring

- Target compliance: **≥ 95%**, measured by electronic monitoring systems and trained secret observers (minimum 200 observations per unit per month).
- Results are posted monthly on unit huddle boards and reported to the Board Quality Committee.

5. HAI surveillance and targets

Measure	Definition	FY2026 target (SIR)
CLABSI	Central line-associated bloodstream infection	< 0.50
CAUTI	Catheter-associated urinary tract infection	< 0.60
SSI	Surgical site infection (colon, hysterectomy, hip, knee, CABG)	< 0.75
MRSA bacteremia	Hospital-onset	< 0.60
C. difficile	Hospital-onset LabID event	< 0.55

Data is reported to CDC's **National Healthcare Safety Network (NHSN)** and is publicly reported by CDPH. SIR = Standardized Infection Ratio.

6. Prevention bundles

- **Central lines:** insertion checklist, chlorhexidine skin prep, maximal sterile barriers, daily necessity review, CHG bathing in ICUs.
- **Urinary catheters:** insertion only for approved indications; nurse-driven removal protocol; bladder scanner first.
- **Surgical site:** pre-op CHG bathing, nasal decolonisation for orthopaedic and cardiac cases, weight-based antibiotic prophylaxis within 60 minutes of incision, normothermia, glucose control.
- **Ventilator-associated events:** head of bed raised ≥ 30°, daily sedation interruption, oral care with a toothbrush every 12 hours.

7. Antimicrobial stewardship

A multidisciplinary Antimicrobial Stewardship Program (ASP), led by an ID physician and an ID pharmacist, does prospective audit and feedback, prior authorisation for restricted agents (e.g. carbapenems, daptomycin), IV-to-oral conversion and annual antibiogram publication. This meets the requirements of CA Health & Safety Code 1288.8.

8. Employee health

- Required immunity and screening: see AMW-HR-001.
- **Exposure to blood or body fluids:** wash immediately, report to Employee Health or the ED within **1 hour**, and post-exposure prophylaxis (PEP) is offered within 2 hours where indicated.
- Staff with fever, vomiting, diarrhoea, draining lesions or a new rash must not work until cleared ("Don't Come To Work Sick" policy, with protected sick leave).

9. Outbreak management

An outbreak is suspected when cases exceed baseline (e.g. 2+ epidemiologically linked cases of the same organism). Infection Prevention will: 1. confirm the case definition and line list 2. implement control measures 3. notify the local public health department (**within 24 hours** for reportable outbreaks under 17 CCR 2500) 4. communicate with staff, patients and families 5. complete an after-action review

10. Construction and renovation

An **Infection Control Risk Assessment (ICRA 2.0)** is needed before any construction, renovation or maintenance that disturbs ceilings, walls or water systems. Water management follows ASHRAE 188 to prevent *Legionella*.

11. Visitors

Visitors with symptoms of infection are asked to postpone. During declared respiratory seasons, masks may be required in patient-care areas. Children under 12 may be restricted from high-risk units (NICU, BMT, ICU) during outbreaks.

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